

ST. JUDE GENERAL HOSPITAL

123 Healthcare Boulevard, Suite 400 • Phone: (555) 019-2831

DISCHARGE SUMMARY

Patient Name:	DOE, JOHN R.	MRN / ID:	MRN-9842011A
DOB / Age / Sex:	10/14/1968 (57 yrs) • Male	Encounter No:	ENC-2026-8812
Admission Date:	March 18, 2026	Discharge Date:	March 22, 2026
Attending MD:	Dr. Sarah Jenkins, MD (Cardiology)	Discharge Status:	Home (Stable)

1. DIAGNOSES & CLINICAL SUMMARY

Primary Discharge Diagnosis: Acute Decompensated Heart Failure (Preserved Ejection Fraction), resolved.

Secondary Diagnoses: Essential Hypertension, Type 2 Diabetes Mellitus, Chronic Kidney Disease (Stage II).

2. HISTORY & HOSPITAL COURSE

Reason for Admission: Patient presented to the ED with a 4-day history of worsening exertional dyspnea, bilateral lower extremity pitting edema (+2), and a 6 lb weight gain over one week.

Hospital Course Summary: The patient was admitted to the Acute Care Medical Unit. Intravenous diuresis was initiated with IV Furosemide 40mg BID, resulting in significant fluid clearance (-4.2 kg net loss over 4 days). Daily electrolyte panels were monitored and potassium supplementation was adjusted accordingly. Oxygen saturation normalized on room air by Hospital Day 3. Echocardiogram revealed an EF of 55% with grade I diastolic dysfunction. Patient is clinically stable for discharge.

3. DISCHARGE PHYSICAL EXAM & VITALS

BLOOD PRESSURE 124 / 78 mmHg	HEART RATE 68 bpm (SR)	SPO2 98% (Room Air)	WEIGHT 81.5 kg (-4.2 kg)
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4. KEY DIAGNOSTIC & LAB RESULTS

Test Description	Admission Value	Discharge Value	Reference Range
NT-proBNP	1,850 pg/mL (H)	420 pg/mL	< 125 pg/mL
Serum Creatinine	1.45 mg/dL	1.18 mg/dL	0.74 – 1.35 mg/dL
Potassium (K+)	3.4 mEq/L	4.2 mEq/L	3.5 – 5.1 mEq/L
Hemoglobin A1c	7.1 %	Not repeated	< 5.7 %

5. DISCHARGE MEDICATION SCHEDULE

Please resume pre-admission medications except where adjusted below:

Medication Name	Dosage & Route	Frequency	Status / Instructions
Furosemide (Lasix)	20 mg Oral	Daily (Morning)	Adjusted: Step-down from IV. Take with food.
Lisinopril	10 mg Oral	Daily	Continued: Blood pressure management.
Metformin HCl	500 mg Oral	Twice Daily	Continued: Take with meals.
Potassium Chloride	10 mEq Oral	Daily	New: Take for 14 days, then recheck serum K+.

6. CARE PLAN & ACTIVITY GUIDELINES

- Dietary Plan:** Low sodium diet (< 2,000 mg/day). Fluid intake restricted to 2.0 Liters per day.
- Weight Monitoring:** Weigh yourself every morning after voiding and before breakfast. Log weights daily.
- Activity Level:** Light activity as tolerated. Avoid strenuous lifting (> 15 lbs) for 1 week.

WHEN TO CALL THE CLINIC OR SEEK IMMEDIATE CARE

Contact the outpatient cardiology clinic or proceed to the nearest emergency department if you experience:

- Weight gain of 3 lbs in 24 hours OR 5 lbs in one week.
- Increased shortness of breath, especially when lying flat or waking at night.
- New or worsening swelling in ankles, legs, or abdomen.
- Chest pain, dizziness, or lightheadedness.

7. FOLLOW-UP APPOINTMENTS

Provider / Specialty	Location / Clinic	Date & Time
Dr. Sarah Jenkins (Cardiology)	St. Jude Heart Center, Suite 200	March 29, 2026 at 10:30 AM
Dr. Robert Chen (Primary Care)	Community Medical Group	April 05, 2026 at 02:00 PM

Dr. Sarah Jenkins, MD
Attending Physician • License #MD-90218

John R. Doe (or Caregiver)
Patient Acknowledgment & Receipt

